Page of Contents

Sl.Order	Particulars
A	Purpose
B	Scope
C	Responsibility
D	Departmental Hierarchy
E	Objectives
F	Emergency Department Classification of Capability & Staffing
G	Emergency Care Services
H	Coverage Area of Hope Hospital
I	Emergency Preparedness Plan
J	Consent for Treatment
K	Patient's Initial Screening Exam
L	Ambulance Services
M	Maintenance of Medical Records
N	Radiology Services and Laboratory Services
O	Admitting Patients from the Emergency Department
P	Transfer of Patients
Q	Medico legal Cases
R	Storage of Medicines in Emergency Department
S	Infection control in Emergency Department

	Training of staff in Emergency department

A. Purpose: - To provide guideline instructions for the provision of immediate relief to and management of the patients arriving at the hospital with acute medical and surgical emergencies with any injuries by accidents, sudden attacks of illness, head trauma, Physical abuse, poisoning, burns and rape cases etc without any discrimination

B. Scope: Scope of services of the ED range from providing episodic, primary, acute (comprehensive) care to referrals.

C. Responsibility: Chief Medical Officer, Emergency staff Nurse

D. Departmental Hierarchy:

Chief Medical Officer
↓
ED Nursing Staff
↓
ED Attendants
↓
Housekeeping Staff.

E. Objectives:

- To triage all incoming patients.
- To have patients assessed by qualified individuals.
- To diagnose, treat, admit and provide appropriate referral and follow up.
- To ensure critically ill patients receive the top priority care as determined by triage guidelines.

- To initiate lifesaving treatment.
- To provide end of life care.

F.Emergency Department (ED) Classification of Capability & Staffing

1. The Emergency Department of Hope Hospital offers comprehensive emergency care 24 hrs a day.
2. One Emergency Medical Officer is on duty in the ED during the morning, evening and night shift respectively.
3. During peak hours, the consultants of most medical services are available in the hospital and can be reached immediately incase of any need.
4. During non peak hours the consultants from each clinical department are available on call basis.
5. In case of Accidents involving numerous individuals at a time all consultants and staff members responsible to provide critical can be called as per the requirement.

G.Emergency Care Services

The ED service covers evaluation, resuscitation and treatment of all the emergency conditions; it involves both pre-hospital and in-hospital emergency services of the following types:

1. Cardio-pulmonary emergencies.
2. Surgical Emergencies
3. Trauma Related Emergencies
4. Medico Legal Emergencies
5. Endocrinal Emergencies
6. Gynecological Emergencies
7. Infectious Emergencies

8. Ambulance Services

ED Services not provided at Hope Hospital:

1. Burns Critical Care (As a dedicated Burns Critical Care Unit is not yet available at Hope Hospital)
2. Obstetrics Emergencies
3. Neonatal Emergencies

H.Coverage area of Hope Hospital

Hope Hospital serves as a secondary referral center for local health clinics. The coverage area of the Emergency Services includes the City limits of Nagpur & beyond. The ambulance service of Hope Hospital covers Nagpur and surrounding places The hospital's Emergency Department patients may also be referred to other hospital emergency departments by following the Inter-Hospital Transfer of Patients Policy in case of non-availability of beds or services after stabilizing the patient in the ED.

I. Emergency Preparedness Plan (Disaster preparedness plan)

1.Response Time

All patients will come to the ED for emergency medical evaluation or treatment will receive care by qualified personnel in a timely manner consistent with the acuity of their illness. Hope Hospital has a policy to attend to the patients arriving in the ED immediately. The Nurse assessment at the triage is done immediately. All patients arriving in the ED are examined and attended by doctors without delay. The Consultants of respective specialty are called & they attend to the patient immediately during the regular hours of operations of the OPD. During after hours, Consultants on call are contacted immediately upon need. Treatment to patients who are critical is initiated immediately without any delay for the purpose of documentation and consent.

2.Triage

Emergency Department patients will receive prompt initial assessment by a registered nurse and will have emergency care initiated according to their level of acuity.

The desired out come of the triage process is that all Emergency Department patients will receive expedient treatment according to established priorities.

Emergeny patients requiring immediate intervention are transferred to the appropriate bed station in the ED to initiate the patient assessment & care process.
The registration process of the patient is also initiated in the ED if the patient condition permits. In case of limb and life threatening situations the registration and consent process are postponed so as to facilitate the initiation of appropriate emergency care.

1. The most severe patients are treated and transported first, while those with lesser injuries are transported later.
2. Decision is made about who will be managed first.
   In a choice between a patient with a catastrophic injury, such as severe open head trauma, a patient with an acute intra abdominal hemorrhage the proper course of action in an Multiple Emergency Incidents (MEI) is to manage first the salvageable patient : - The one with the abdominal hemorrhage. Treating severe head injury patients first probably will cause loss of both the patients. As it is not salvageable the abdominal hemorrhage patient because of time, equipment and personnel spent managing the unsalvageable patient. Keep the salvageable patient from getting simple care that are almost certainly keep her alive long enough to reach definite surgical cost.

3. The following "Sorting Scheme" is used in the ED for prioritizing the emergency patient care according to the acuity of the patient's condition:

1. Immediate: Those patients whose injuries are critical but who will require minimal time or equipment to manage and who have a good progress for survival. E.g.:- patient with a compromised airway or massive external hemorrhage.

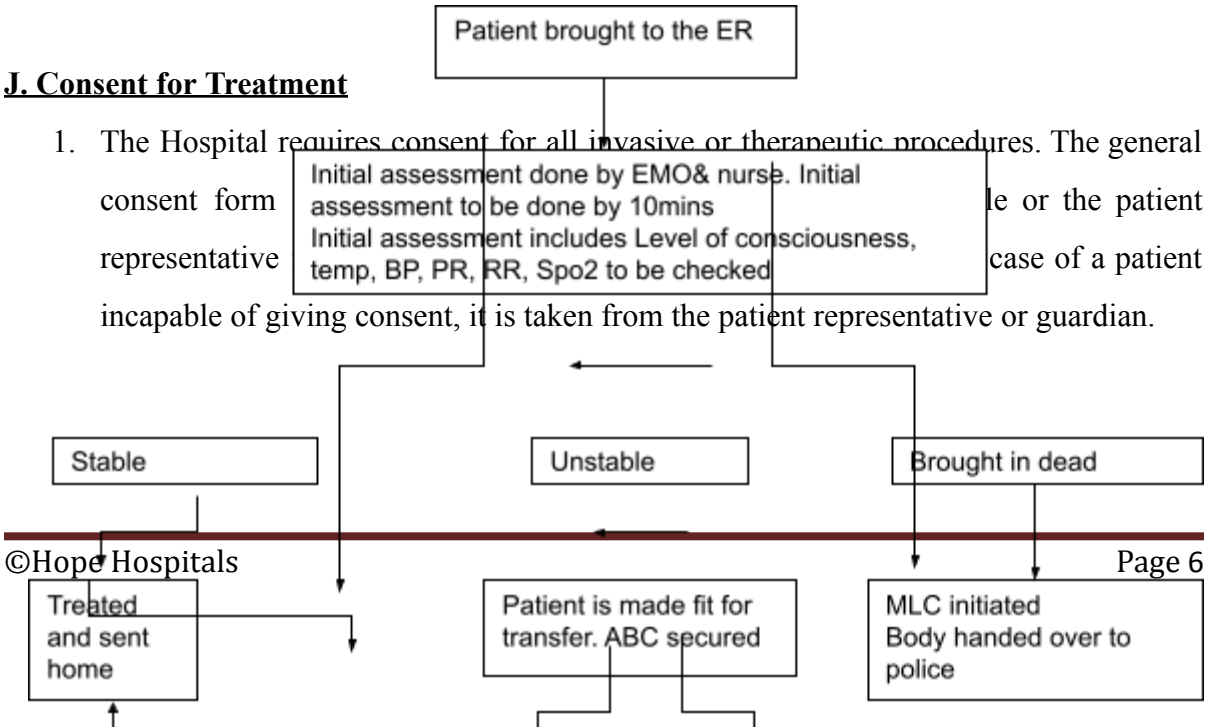
2. Delayed: Those patients whose injuries are debilitating but who do not need immediate management to salvage life or limb. E.g.:- Long Bone fracture

3. Expectant: - Whose injuries are so severe that they have only a minimal chance of survival. E.g.:- Patient with 90% full thickness, burns are thermal pulmonary injuries.

4. Minimal: - Who have minor injuries that can wait for treatment are who may even assist in the intern by comforting other patients.

5. Dead: - Who is unresponsive, pulse less, Breathless, in a disaster, resources rarely allow for attempted resuscitation.

Triage Decisions



J. Consent for Treatment

1. The Hospital requires consent for all invasive or therapeutic procedures. The general consent form le or the patient representative case of a patient incapable of giving consent, it is taken from the patient representative or guardian.

2. Life-sustaining measures are not withheld for lack of formal consent if there is no time to obtain the consent for urgent procedures. The consent process is postponed and treatment is started immediately in such cases.
3. Consent is required for elective blood transfusions that are not life threatening.

K. Patient Initial Screening Exam

1. The initial assessment will be done by the ED EMO/ nurse for emergency patients.
2. The time frame for the initial assessment will be 10 minutes.
3. The Initial assessment will include ascertaining the level of consciousness, checking the blood pressure, Pulse, temperature, Spo2, GRBS in case of diabetics.
4. The initial assessment will ascertain the condition of the patient whether stable or unstable and appropriate measures will be taken.
5. Initial Assessment will include nutritional assessment of patient
6. initial assessment by the medical officer will include the following criteria:
   a. **Assessment criteria for non Road Traffic Accident patients include:**
      i. Presenting History:
      ii. Past Medical History:
      iii. Allergies:
      iv. O/E:
         - Temp. ,BP , PR, Spo2-, GRBS(optional),
      v. CVS/RS/ABD/CNS:
      vi. Investigations done:
      vii. Provisional diagnosis:
      viii. Treatment given:
      ix. Course of action: outpatient/admission/transfer out/references
   b. **Assessment criteria for Road Traffic Accident patients include:**
      i. Presenting history:

ii. Past medical history:

iii. Allergies:

iv. Last meal:

v. O/E:

- Level of consciousness- , GCS, Pupils, Temp-, BP- ,PR

vi CVS/RS/ABD/CNS:

vii.L/E:

viii.Investigations done:

ix.Provisional diagnosis:

x.Treatment given:

xi.Course of action: outpatient/admission/transfer out/references

xii.MLC initiated

7. The initial assessment will result in documented plan of care.

L. Ambulance Services

Please refer to the Ambulance Services Document

M.Maintenance of Medical Records (Registers and Documents maintained)

The following records are maintained in the ED:

1. List of Consultants on Duty (During Peak Hours) and on call (during non peak hours)
2. Case files of patients attended in the ED
3. MLC register for medico legal cases
4. Drug Inventory Register
5. Controlled Drugs and Psychotropic Drugs register
6. Brought Dead Certificate
7. Death Certificate

N.Radiology Services & Laboratory Services

The ER of Hope Hospital is equipped for undertaking all essential lab investigations and radiological work up for the patient , it collaborates with the laboratory and imaging department to provide such services on an emergency basis. The hospital also has a portable X-ray and Ultrasound machine to conduct the examinations at the bedside in the ER. After the necessary investigations are ordered, results are obtained from the laboratory by phone in cases urgency. When certain investigations like Blood Toxicology and Arterial Blood Gases which are not conducted at our in house laboratories are required, these tests are outsourced to outside laboratories.

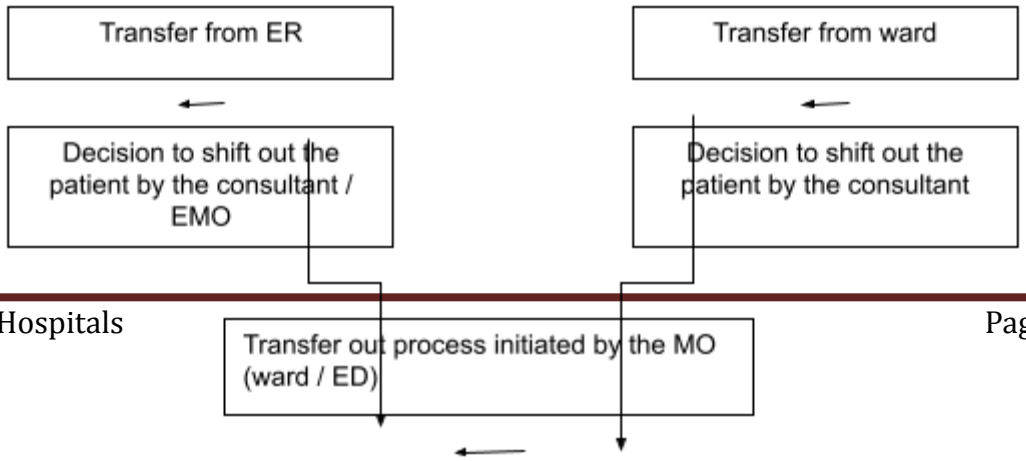
O.Admitting Patients from the Emergency Department

1. In case admission of the patient is necessary, the EMO / Consultant on duty make the decision for admission and authorize it. The EMO admits the patient under the specialty Consultant on duty (during peak hours) and on call basis (during non peak hours).
2. The ED nurse is informed if the patient is to be admitted.
3. Admission to the ICU is approved by the attending Consultant.
4. After the patient representative makes the necessary admission procedure & admission is confirmed, necessary arrangements are made to transfer the patient to the floor by the ED nurse staff on duty in collaboration with the housekeeping staff.

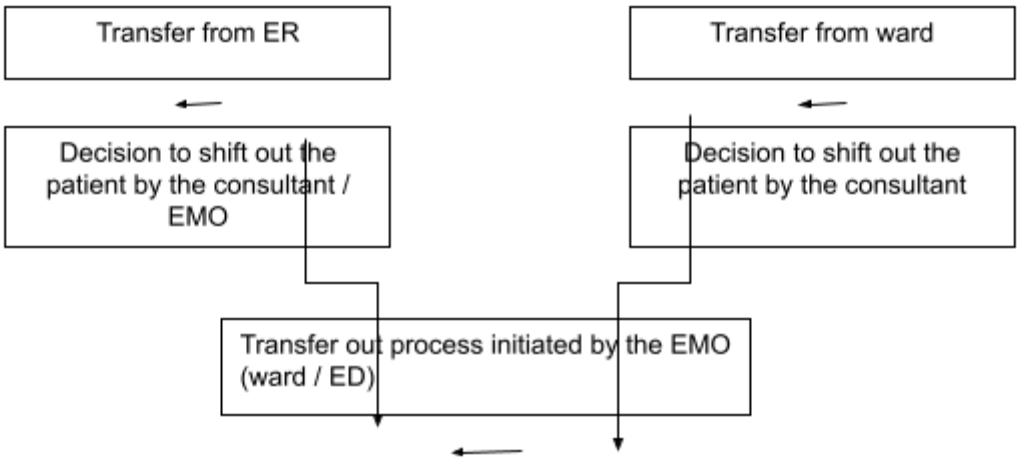
5. The ED nurse communicates with the nurse in charge of the floor and confirms the availability of the bed and initiates the transfer of the patient to the floor admitted.
6. Patient is transferred to the floor by transport by the housekeeping staff as per patient's acuity. Monitored patients are transferred with a Nurse. All documents and reports of the patient are transferred to the floor along with the patient.
7. Exceptions occur in cases of life and death emergencies. The patient will be transferred to the ICU directly from the ED and registration & documentation may be postponed.

P.Transfer of patient:

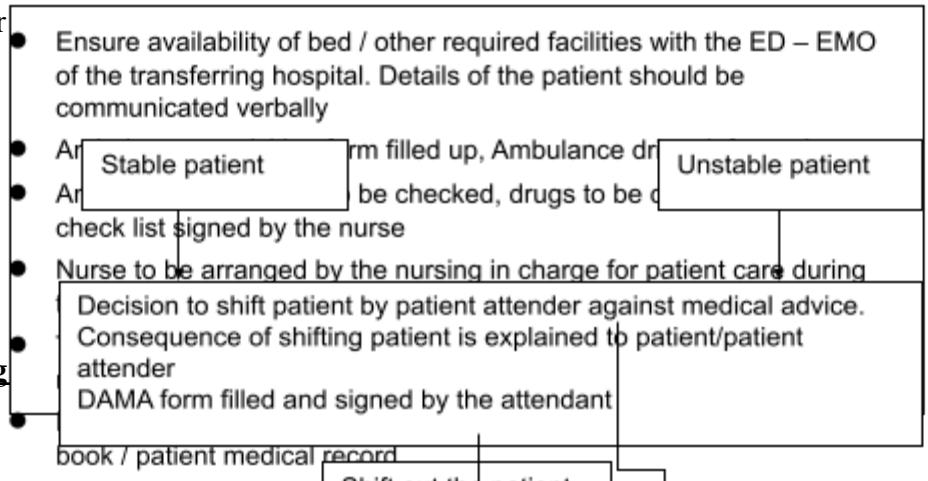
1. Transfer out of stable patients from ED/Ward (at request /non availability of Facilities)



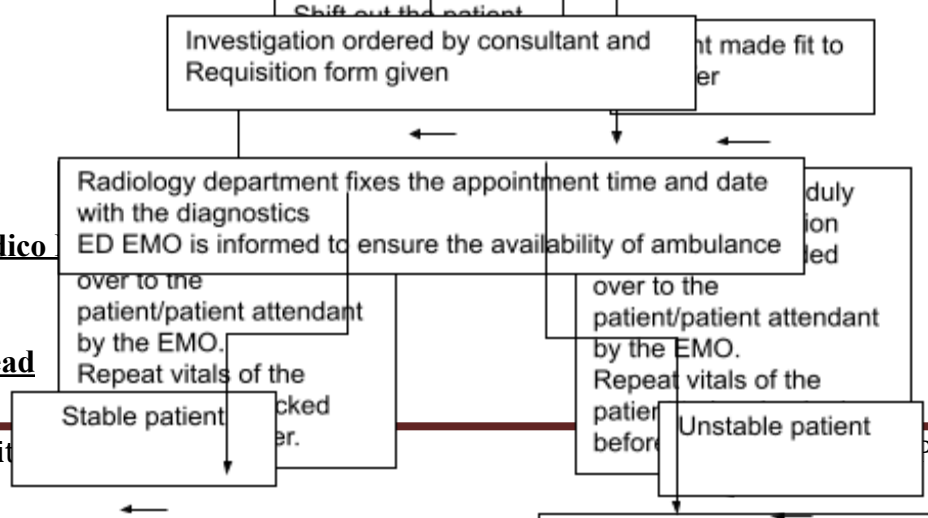
2. **Transfer of unstable patient from ED/ward (on request /non availability of services)**



3. Transfer



4. **Shifting**



Q.MLC (Medico

1.Brought Dead

i. Take past history – HTN / DM / IHD etc.,

ii. Look for / Ask about any suspicious signs:

- Poisoning – Smell
- Strangulation – Ligature mark around neck / abnormal sings
- Any external injuries
- Expose the body completely and look for any sings
- Palpate the head and look for any haematoma, etc which may be missed.

iii. If a female, ask history of married life and if it is less than 7 years register it as MLC, - it is mandatory.

Register all brought dead cases as medico-legal case if death has occurred unexpectedly or from an unexplained cause.

On arrival, the Emergency Medical officer should examine the patient thoroughly. He / She should go into the history in detail and look for signs of homicide, suicide, violence, external injuries to rule out any suspicious cause for the death. In case of female patient, marital history should be elicited and if EMO feels suspicious cause for the death, Medico Legal Case has to be registered.

After complete examination and confirmation by clinical evaluation death & is confirmed, the individual should be declared as Brought in Dead (BID) and the accompanying relatives/friends must be explained and informed about the probable cause of death and they are given only a Brought Dead Certificate until the cause of death is confirmed. The local police (Dhantoli) should be informed immediately in case suspicion or foul play. The police will do the further disposal of the dead body after inquest. The Medical Officer will render necessary assistance.

2. Death on Arrival:

If a patient has sudden Cardio-Respiratory Arrest on arrival at the Emergency Room, the patient has to be resuscitated as per ACLS protocols (Ref. Document). Once death is confirmed the case should be treated as death on arrival, and necessary documentation should be done. EMO should go into the detailed history of the patient and arrive at the probable cause of death. On the basis of this, death certificate should be issued and arrangements for release of the body are taken after settlement of hospital dues.

3. Handling of Death & Release of Dead Body

Death of a patient is handled carefully with concern without complacency. Counseling of next of kin with sympathy is given at most importance. All help in shifting the body from the hospital is extended to the next of kin. The dead body is released as soon as possible after completion of all formalities.

Acknowledgement for receipt of the body and the Death Certificate is obtained from Next of Kin/Legal representative. Handing-over of the body is a solemn occasion and it is ensured that hospital staff takes due care and concern in this respect. Due arrangements are made if preserving the body in the mortuary is found necessary.

A security staff of the hospital is present till the departure of the deceased and ensures orderliness in handing over the body to the next of kin.

4. Death Certificate:

EMO should certify the cause of death in the Death Certificate after careful and thorough examinations of the patient after discussing with the concerned consultant. Death certificate is initiated if the death occurs within the hospital, unless there are grounds and evidence to the contrary. The cause of death should be well documented and a copy of the Death certificate should be filed along with the medical documents of the deceased patient.

R.Storage of Medicines in Emergency Department

1. All Emergency medications will be available 24 hrs in the ER (refer list of emergency medication)
2. All Emergency medications will be replenished by the nurse/pharmacist on duty with each case and on daily basis.
3. Medication inventory / Crash cart will be checked by the nurse on duty with each shift change, to detect loss or theft.
4. Narcotics drugs will be prescribed and procured from the nearest pharmacy other than Hope Pharmacy and will be administered under the supervision of the nurse in Charge.
5. Prescription of Narcotic drugs will be released only on the signed requisition of the consultant/MO.
6. Working condition of the ER equipments will be checked by the nurse on duty with each change in shift.

7. Any Malfunction /nonfunctioning of the equipment will be brought to the notice of the nurse in charge and the Chief medical officer and work order is raised.

S.Infection Control In ED

1. All Emergency Medical officers will undergo training on infection control
2. All Emergency Medical officers will follow the infection control procedures as laid down by the infection control Committee.
3. All Needle prick injuries will be reported through incident report to the chief medical officer
4. Screening for MRSA will be done in the ED for all patients who are transferred in from other hospital with History of 48hrs and above stay in that hospital .screening will also be done for bedridden patients.
5. Swabs will be taken from the nose, axilla, groin, bedsores (if present) of patients fulfilling those criteria and sent to lab and will be informed to the Respective unit nurse on handing over the patient.
6. Since ED is one of the high risk areas standard precautions will be taken by the staff at all times.

7. Equipment cleaning and sterilization will be supervised by the nurse in charge
8. Swabs will be taken from the different areas and will be screened for nosocomial pathogens.

9. Swabs will be taken once in 30 days and follow up of the report will be done by the nurse in charge

T. Training of Staff on the procedures of care of emergency patient:

The staff is trained regularly on the following topics

Procedures mentioned in the policy 101COP to 132COP in COP folder

It will also include procedures mentioned below but not limited to

Chest tube insertion
Cricothyrotomy
Defibrillation
Intubation
Laryngoscopy
Tracheostomy

EMERGENCY ROOM ASSESMENT FORM

x. **Presenting History:**

xi. **Past Medical History:**

xii. **Allergies:**

xiii. **O/E:**

- Temp
- BP

- PR
- Spo2-
- GRBS(optional)

xiv. CVS

xv. RS

xvi. ABD

xvii. CNS:

xviii. **Investigations**

xix. **Provisional diagnosis:**

xx. **Treatment given:**

xxi. **Course of action:**

outpatient/admission/transfer out/references/MLC intimitation

Doctor on duty - sign date /time